

JOHNS HOPKINS MEDICINE
NEUROLOGY CENTRAL INTAKE FAX
FAX RECEIVED: 03/18/2026 14:22
PAGES: 17 (INCLUDING COVER)

Referral Cover Sheet
Referring Office: Centreville Family Practice Associates
Provider: Anil K. Sharma, MD
Phone: (703) 555-2187
Fax: (703) 555-2199

Patient Name: Lila M. Carver
DOB: 08/11/1962
Reason for Referral: chronic headaches / facial pain (rule out atypical migraine vs neuropathy vs sinus?)

Requested Service: Neurology consult (headache clinic?)

Clinical Notes:
Patient with severe intermittent right facial pain x ~1 yr, worsening. Multiple ED visits. Poor response to triptans. MRI done (see attached). Some dental work previously. Please evaluate.

Insurance: Aetna PPO
Urgency: Routine (pt reports severe pain but stable per last visit)

Handwritten addendum: "pain is electric? not sure. pls eval."

Page 1 of 17

PATIENT DEMOGRAPHICS
Printed: 03/18/2026 13:58

Name: Carver, Lila Mae
MRN: 00488219
DOB: 08/11/1962
Age: 63
Sex: F
Address: 14219 Willow Bend Ct, Centreville, VA 20120
Phone: (571) 555-8821
Emergency Contact: Daniel Carver (spouse) (571) 555-8822

Primary Care Provider: Sharma, Anil K MD

Insurance: Aetna Choice POS II
Member ID: W123449882
Group: 88321

Preferred Pharmacy: CVS #3298 Centreville

Allergies:

NKDA (note: later entry lists “possible rash w/ penicillin?” unclear)

Preferred Language: English

Smoking Status: Former (quit 2015 per intake) / Current occasional (per 2025 note)

Page 2 of 17

REFERRING PROVIDER NOTE

Date: 03/12/2026

Provider: A.K. Sharma MD

Chief Complaint: “face pain again”

HPI:

63F with history of HTN, GERD, anxiety, ?migraine presents for follow-up of right-sided facial pain. Reports sharp, stabbing episodes lasting seconds to minutes, occurring multiple times daily. Triggered by brushing teeth, cold air, talking sometimes. She describes as “electric shock” but also “pressure behind eye.”

Has tried sumatriptan without benefit. OTC NSAIDs minimal relief. Was treated for sinusitis twice in past year with antibiotics (amoxicillin, then doxycycline) with no sustained improvement. Seen by dentist, underwent extraction R upper molar approx 8 months ago without resolution.

Pain sometimes wakes her at night (inconsistent history—prior notes state daytime only).

Denies visual aura. Occasional tearing R eye but no consistent rhinorrhea. No nausea/vomiting with episodes (though prior note says nausea?).

Past imaging: MRI brain 11/2025 (see report).

Assessment:

Chronic facial pain—atypical migraine vs trigeminal neuropathy vs cluster variant?

Plan:

Refer to tertiary neurology for further eval

Continue gabapentin 300 mg TID (patient unsure if helping)

Trial carbamazepine discussed but not started due to patient concern about side effects

Follow up after consult

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Page 3 of 17

PRIOR NOTE #1

Emergency Department Visit

Facility: Inova Fair Oaks Hospital

Date: 12/04/2025

Chief Complaint: Headache

HPI:

62 yo female presents with acute severe right-sided facial pain radiating from jaw to temple. Sudden onset while eating lunch. Describes as 10/10 “shooting” pain lasting seconds, recurring every few minutes. Reports similar episodes over past several months but today worse.

Denies fever, neck stiffness. Denies photophobia. No focal weakness.

PMH: HTN, GERD, “migraines” (unclear)

Meds: lisinopril, omeprazole, sumatriptan PRN

Allergies: NKDA

Exam:

Neuro nonfocal. Cranial nerves intact though patient hesitant to allow facial palpation due to pain.

ED Course:

Given ketorolac IV with minimal relief. Pain episodic, not continuous. Neurology consulted by phone—recommend outpatient follow-up.

Discharge Dx: atypical headache vs neuralgia

Discharge Meds:

Gabapentin 100 mg TID started

Instructions: follow up PCP

Page 4 of 17

PRIOR NOTE #2

ENT Clinic Visit

Date: 09/18/2025

Provider: Laura Chen, PA-C

Reason: chronic sinus pressure

HPI:

Patient reports 6+ months of right facial pressure, worse in maxillary region. Associated with intermittent nasal congestion. Pain described as dull ache with occasional sharp flares. Worse with bending forward.

Denies significant dental issues (note: later dental extraction documented elsewhere).

Exam:

Nasal mucosa mildly edematous. No purulence.

Assessment: chronic sinusitis vs atypical facial pain

Plan:

CT sinuses ordered

Amoxicillin-clavulanate x 14 days

Flonase

Addendum (10/02/2025):

CT sinuses essentially normal. Symptoms persist. Consider neurology referral.

Page 5 of 17

PRIOR NOTE #3

Primary Care Visit

Date: 07/02/2025

Provider: A.K. Sharma MD

Chief Complaint: headaches

HPI:

Patient reports episodic right-sided headaches for several months. Describes throbbing at times, sometimes sharp. Occur a few times per week, lasting minutes to hours (inconsistent with later reports of seconds).

Associated with nausea per patient today. Light sensitivity unclear.

Assessment: migraine variant

Plan:

Start sumatriptan PRN

Lifestyle modification

Copy-forward text:

“Patient with longstanding migraine history since her 30s” (no documentation elsewhere confirming this)

Page 6 of 17

LAB RESULTS

Collected: 11/28/2025

CBC:

WBC 6.2

Hgb 13.4

Plt 245

BMP:

Na 139

K 4.1

Cr 0.8

ESR: 12 (normal)

CRP: 0.9

Vitamin B12: 412

TSH: 2.1

Lyme Ab: negative

Page 7 of 17

IMAGING REPORT

MRI BRAIN W/VO CONTRAST

Date: 11/30/2025

Indication: chronic headache, facial pain

Findings:

No acute infarct, hemorrhage, or mass. Ventricles normal.

Note: small vessel ischemic changes mild.

Cranial nerves: limited evaluation. No obvious mass lesion along expected course of trigeminal nerve though dedicated protocol not performed.

Vascular: possible vascular loop near right trigeminal root entry zone vs artifact (not well characterized).

Impression:

No acute intracranial abnormality.

Consider dedicated cranial nerve imaging if clinically indicated.

Page 8 of 17

CT SINUSES

Date: 09/30/2025

Findings:

Clear paranasal sinuses. No mucosal thickening.

Impression: normal CT sinuses

Page 9 of 17

MEDICATION LIST (ACTIVE)

Printed 03/18/2026

Lisinopril 20 mg daily

Omeprazole 20 mg daily

Gabapentin 300 mg TID (patient reports taking "sometimes")

Sumatriptan 50 mg PRN (ineffective per pt)

Ibuprofen 400 mg PRN

Vitamin D 1000 IU daily

Multivitamin daily

Sertraline 50 mg daily

Melatonin 5 mg nightly

Cyclobenzaprine 5 mg PRN (from prior back pain episode)

Old meds still listed:

Doxycycline (completed)

Amoxicillin-clavulanate (completed)

Page 10 of 17

DENTAL NOTE (SCANNED)

Date: 10/22/2025

Procedure: extraction upper right molar (#3)

Indication: suspected dental origin of pain

Note: patient reported shooting pain triggered by chewing. Tooth with mild decay but unclear if source.

Post-procedure: pain persists per patient phone call 1 week later

Handwritten: "may not be dental etiology"

Page 11 of 17

URGENT CARE VISIT

Date: 01/14/2026

Chief Complaint: facial pain

HPI:

Patient states "lightning bolt pain" right cheek triggered by washing face. Lasts seconds but repeats. Reports avoiding brushing teeth due to fear of pain.

Exam:

Normal vitals. No swelling. Neuro intact.

Assessment: neuralgia?

Plan: increase gabapentin to 300 mg TID

Page 12 of 17

DISCHARGE SUMMARY (SHORT STAY OBSERVATION)

Facility: Inova Fair Oaks

Admit: 02/03/2026

Discharge: 02/04/2026

Reason: uncontrolled facial pain

Hospital Course:

Patient admitted for pain control. Episodes brief but severe. No response to opioids. Trial of carbamazepine discussed but deferred pending outpatient follow-up.

Neurology consult note (brief):

"Consider trigeminal neuralgia vs atypical facial pain. Recommend outpatient specialty evaluation."

Discharge Meds:

Gabapentin continued

Oxycodone PRN (patient later states she did not take)

Page 13 of 17

VITALS SUMMARY

Various dates

BP range: 128/76 – 152/88

HR: 68–84

Weight: 168 lb

Page 14 of 17

PROBLEM LIST

Hypertension

GERD

Anxiety

Headache disorder

Facial pain

Chronic sinusitis (questionable)

Page 15 of 17

NURSING INTAKE NOTE (TRUNCATED)

Date: 03/12/2026

Pain score: 8/10

Location: right face

Description: sharp, stabbing, electric

Triggers: talking, brushing teeth, wind

Duration: seconds

Patient tearful during intake

Page 16 of 17

FAX FOOTER

Centreville Family Practice

Fax transmitted successfully

03/18/2026 14:21

End of transmission

Page 17 of 17

